

Media Information



Report Status: Final - Courtesy Copy

MALASI, MARJETA

Patient Information	Specimen Information	Client Information
MALASI, MARJETA DOB: [REDACTED] Gender: F Fasting: U Phone: [REDACTED] Patient [REDACTED]	Specimen: CF096534V Requisition: 7899152 Collected: [REDACTED] / 14:00 EDT Received: 08/19/2025 / 02:18 EDT Faxed: 08/25/2025 / 16:48 EDT	Client #: 48039280 NYNJMAIL KASHANIAN, FARZAM FARZAM KASHANIAN,MD 2350 OCEAN AVE STE 4 [REDACTED]

COMMENTS: ADD ON

Test Name

ER/PR, PARAFFIN BLOCK
HISTOPATH TISSUE/BIOPSY

see note

AND

TESTS: Processed Specimen Quantity (1)
HER2, IHC WITH REFLEX TO FISH (1)

CLINICAL DIAGNOSIS / INFORMATION:

Interpretation of Submitting Facility: Infiltrating ductal carcinoma

SITE OF BIOPSY / SOURCE:

Block(s) - CT25-8077086-A2 Breast, right, retro areolar, core needle biopsy (1 blk)

GROSS DESCRIPTION:

Received is one paraffin-embedded tissue block labeled CT25-8077086-A2.

MICROSCOPIC DESCRIPTION:

The patient [REDACTED] [REDACTED] [REDACTED]
monoclonal antibody, using a polymer based technique and DAB as a
chromogen. Controls stain appropriately unless otherwise noted.

Specimen Adequacy: Adequate for evaluation.

Type of Fixation: Formalin

Duration of Fixation: 6-48 hours

Type of tissue processing: Routine

Cold ischemic time: Not provided

Estrogen Receptor (Dako clone ER1)

Scoring Guide: Positive: >10% / Low Positive: 1-10% / Negative: <1%

Progesterone Receptor (Dako clone PgR636)

Scoring Guide: Positive: $\geq 1\%$ / Negative: $<1\%$

HER2/neu (cerbB-2) immunohistochemistry, Ventana Pathway 4B5.

Specimen Adequacy: Adequate for evaluation

Internal and External controls: Stain appropriately.

There is no membrane staining of cells in the lesion.

----- FINAL DIAGNOSIS -----

SOURCE: BREAST, RIGHT, RETRO AREOLAR, CORE NEEDLE BIOPSY

ESTROGEN RECEPTOR (IHC): POSITIVE: >95%, STRONG INTENSITY

PROGESTERONE RECEPTOR (IHC): POSITIVE: 95%, STRONG INTENSITY

HER2: NEGATIVE FOR OVER-EXPRESSION, SCORE 0 - NO

COMMENTS:

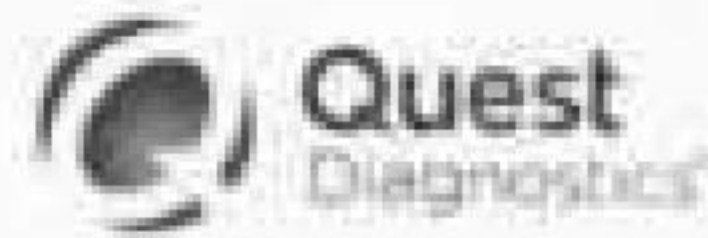
In the DESTINY-Breast 04 and 06 trials, "HER2 Low" was considered IHC score 1+ or 2+/ISH negative, and "HER2 Ultra-Low" was HER2 IHC Score

CLIENT SERVICES: 866.697.8378

SPECIMEN: CF096534V

PAGE 1 OF 4

Quest, Quest Diagnostics, the associated logo and all associated Quest Diagnostics marks are the trademarks of Quest Diagnostics.



Report Status: Final - Courtesy Copy

MALASI, MARJETA

Patient Information	Specimen Information	Client Information
MALASI, MARJETA DOB: 04/05/1978 AGE: 47 Gender: F Fasting: U Patient ID: NG	Specimen: CF096534V Collected: [REDACTED] / 14:00 EDT Received: 08/19/2025 / 02:18 EDT Faxed: [REDACTED]	Client #: 48039280 KASHANIAN, FARZAM

Test Name

Lab

0 (pattern 0+) with membrane staining that is incomplete and faint/barely perceptible in less than or equal to 10% of tumor cells. Breast cancers with these staining patterns may be eligible for treatment with trastuzumab-deruxtecan in the metastatic setting (but those with no [REDACTED] [REDACTED] [REDACTED] [REDACTED] [REDACTED] [REDACTED])

*The Ventana Pathway anti-HER2/neu (4B5) is a semi-quantitative immunohistochemical assay to determine HER2/neu over-expression in breast cancer tissues routinely processed for histologic evaluation. This test has been approved by the FDA as an aid in the assessment of patients for whom Herceptin (trastuzumab) treatment is being considered. The kit used in the staining has been FDA approved for breast and gastric cancers. As with any IHC type assay, a false negative result is possible. A negative result does not exclude the possibility of HER2/neu over-expression. If clinically warranted, more sensitive DNA based methodology (i.e. FISH) for evaluating gene amplification may be indicated.

This assay is performed in concordance with CAP/ASCO criteria. The results of the test can be altered/affected by variables beyond our laboratory's control, e.g. fixatives other than formalin and fixation times other than those recommended in the CAP/ASCO guidelines. The reporting ranges have been modified from the original FDA approved reporting criteria to reflect those recommended in the updated CAP/ASCO guidelines. This test is not intended as a confirmation of the original diagnosis and should be used in conjunction with other established risk factors, clinical and diagnostic procedures.

Patients with breast cancers that are HER2 IHC 3+ or IHC 2+/ISH amplified may be eligible for several therapies that disrupt HER2 signaling pathways. Invasive breast cancers that test 'HER2-negative' (IHC 0, 1+ or 2+/ISH not-amplified) are more specifically considered 'HER2-negative for protein overexpression/gene amplification' since non-overexpressed levels of the HER2 protein may be present in these cases. Patients with breast cancers that are HER2 IHC 0 (incomplete faint membrane staining within $\leq 10\%$ of the tumor cells), 1+, or IHC 2+/ISH not amplified may be eligible for a treatment that targets non-amplified/non-overexpressed levels of HER2 expression for cytotoxic drug delivery.

HER2 Reporting Guidelines per ASCO and CAP:

Negative (Score 0): No [REDACTED] [REDACTED] [REDACTED] [REDACTED] [REDACTED] [REDACTED] incomplete and is faint/barely perceptible and within $\leq 10\%$ of tumor cells.

Negative (Score 1+): Incomplete membrane staining that is faint/barely perceptible and within $>10\%$ of tumor cells.

Equivocal (Score 2+): Weak to moderate complete membrane staining in $>10\%$ of tumor cells or Complete membrane staining that is intense but within $\leq 10\%$ of tumor cells

Positive (Score 3+): Complete membrane staining that is intense and $>10\%$ of tumor cells.

REFERENCES

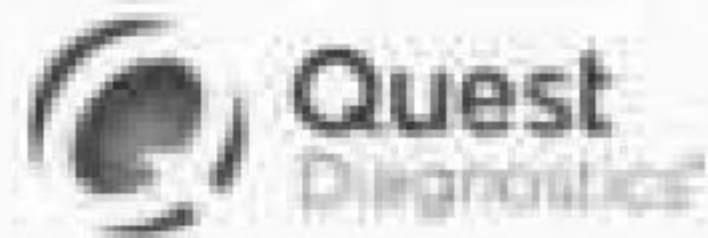
Wolff AC, Hale Hammond ME, Allison KH et al. Human Epidermal Growth Factor Receptor 2 Testing in Breast Cancer Arch Pathol Lab Med. doi: 10.5858/arpa.2018-0902-SA

CLIENT SERVICES: [REDACTED]

SPECIMEN: CF096534V

PAGE 2 OF 4

Quest, Quest Diagnostics, the associated logo and all associated Quest Diagnostics marks are the trademarks of Quest Diagnostics.



Report Status: Final - Courtesy Copy
MALASI, MARJETA

Patient Information	Specimen Information	Client Information
MALASI, MARJETA DOB: [REDACTED] Gender: F Fasting: U Patient [REDACTED]	Specimen: CF096534V Collected: 08/19/2025 / 14:00 EDT Received: [REDACTED] / [REDACTED] EDT Faxed: [REDACTED] / 16:48 EDT	Client #: 48039280 KASHANIAN, FARZAM

Test Name

Wolff AC, Hammond ME, Hicks DG, et al. Recommendations for human epidermal growth factor receptor 2 testing in breast cancer. American Society of Clinical Oncology/College of American Pathologists Clinical Practice Guideline Update JCO Nov. 1, 2013 vol. 31 no. 3997-4013.

Reference:

Allison KH, Hammond EH, Dowsett M, et al. Estrogen and Progesterone Receptor Testing in Breast Cancer: American Society of Clinical Oncology/College of American Pathologists Guideline Update Arch Pathol Lab Med. 2020; 144(5):545-63

Hammond ME, Hayes DF, Dowsett M, et al. American Society of Clinical Oncology/College of American Pathologists guideline recommendations for immunohistochemical testing of estrogen and progesterone receptors in breast cancer. Arch Pathol Lab Med. 2010;134(6):907-922.

Signature on File

Ryan E. Glass, M.D.
Pathologist
[REDACTED]

RESULTS RECEIVED

Reference lab accession: [REDACTED]
SAMPLE FIXED IN FORMALIN? Y
FIXED BETWEEN 6-72 HRS? Y
ROUTINE TISSUE PROCESSING Y
HER 2, IHC W/REFL TO FISH
HISTOPATH TISSUE/BIOPSY see note

AMD

Additional billing charges added.
RESULTS RECEIVED
Reference lab accession: [REDACTED]
SAMPLE FIXED IN FORMALIN? Y
FIXED BETWEEN 6-72 HOURS? Y

CLIENT SERVICES: 866.697.8378

SPECIMEN: CF096534V

PAGE 3 OF 4

Quest, Quest Diagnostics, the associated logo and all associated Quest Diagnostics marks are the trademarks of Quest Diagnostics.



Report Status: Final - Courtesy Copy
MALASI, MARJETA

Patient	Specimen Information	Client Information
MALASI, MARJETA DOB: Gender: F Fasting: U Patient	Specimen: CF096534V Collected: / 14:00 EDT Received: 08/19/2025 / 02:18 EDT Faxed: / 16:48 EDT	Client #: 48039280 KASHANIAN, FARZAM

TISSUE PATHOLOGY**RESULTS****DIAGNOSIS:**

Lab: Z99

A) Breast, right, retro areolar, core needle biopsy:

INFILTRATING DUCTAL CARCINOMA, ARCHITECTURAL GRADE 3, NUCLEAR GRADE 3, MITOTIC RATE 1 (NOTTINGHAM COMBINED HISTOLOGIC GRADE II/III), MEASURING UP TO 0.9 CM IN GREATEST LENGTH, ON A SINGLE CORE

B) Breast, right, 11 o'clock, core needle biopsy:

INFILTRATING DUCTAL CARCINOMA, ARCHITECTURAL GRADE 3, NUCLEAR GRADE 3, MITOTIC RATE 1 (NOTTINGHAM COMBINED HISTOLOGIC GRADE II/III), MEASURING UP TO 0.6 CM IN GREATEST LENGTH, ON A SINGLE CORE

C) Breast, left, 2 o'clock, core needle biopsy:

BENIGN BREAST TISSUE WITH STROMAL FIBROSIS

REPORT NOTES: Key portions of this case have been reviewed by one or more additional department pathologists. Immunohistochemical stains for ER, PR and HER2 will be performed and reported separately.

Marina Cardillo, MD, (electronic signature)
Board Certified in Anatomic Pathology
Pathologist

For questions contact Anatomic Pathology Client Services at and reference DEPT ID: CT258077086

CLINICAL INFORMATION

A: Mass
B,C: Node

SPECIMEN DATA**GROSS DESCRIPTION:**

Lab: Z23

A) Specimen is received in 10% Neutral Buffered Formalin, labeled right breast retro-and with two patient cylindrically-shaped light tan soft tissue ranging from 1.0 x 0.2 to 1.5 x 0.2 cm. The specimen is entirely submitted in cassette(s) A1-A2.

Summary of Sections:

A1-A2 = 2

B) Specimen is received in 10% Neutral Buffered Formalin, labeled right breast 11:00 and with two patient cylindrically-shaped light tan soft tissue ranging from 1.0 x 0.2 to 1.2 x 0.2 cm. The specimen is entirely submitted in cassette(s) B1-B2.

Summary of Sections:

B1 = 2

B2 = 1

C) Specimen is received in 10% Neutral Buffered Formalin, labeled left breast and with two patient shaped light tan soft tissue measuring 0.6 x 0.2 and 1.0 x 0.2 cm. The specimen is entirely submitted in cassette(s) C1.

Grossing and technical component for all specimen parts performed at Ameripath East, Clifton, 1 Insights Drive, Suite A, Clifton, NJ 07012. Laboratory Director: Elaine R. Alt M.D. CLIA No:

PERFORMING SITE:

AMD QUEST DIAGNOSTICS/NICHOLS CHANTILLY, 14225 NEWBROOK DRIVE, CHANTILLY, VA Laboratory Director: PATRICK W. MASON, MD, PhD, CLIA: 49D0221801
Z23 AMERIPATH EAST AMERIPATH NY LLC DBA AMERIPATH EAST, 1 INSIGHTS DRIVE SUITE A, CLIFTON, NJ Laboratory Director: ELAINE R. ALT, MD, CLIA: 31D2100446
Z99 QUEST DIAGNOSTICS CLIFTON, 1 INSIGHTS DRIVE, CLIFTON, NJ 07012-2355 Laboratory Director: DR. PETER L. FISHER, CLIA: 31D0696246

CLIENT SERVICES:

SPECIMEN: CF096534V

PAGE 4 OF 4

Quest, Quest Diagnostics, the associated logo and all associated Quest Diagnostics marks are the trademarks of Quest Diagnostics.



Quest Diagnostics
Anatomic Pathology
1 Insights Drive
Clifton, NJ [REDACTED]
Phone: [REDACTED]
Fax: [REDACTED]

Pathology Material Delivery Receipt
(to be used for overnight or express delivery)

Anatomic Pathology Material related to the patient [REDACTED] of treatment and care of this patient. [REDACTED] therefore, their prompt return within 3 weeks is required without markings or alteration and in the same manner as received. We also request a COPY of any report and notification if the material is forwarded to any other entity.

Patient [REDACTED]

Age/DOB: [REDACTED]

Anatomic Pathology
Material Included: CF096534V-CT258077086/ 5 slides
(Accession [REDACTED])

Requestor's Name: [REDACTED]

Requestor's Phone#: [REDACTED]

Date Sent: [REDACTED]

Delivery Address: MSKCC
ATTN: BRSSL TEAM
300 EAST 66TH ST RM 426
NEW YORK, NY [REDACTED]

Document Information

Other: Outside Pathology

Outside pathology report

00:00

Attached To:

OTHER ORDERS

Orders Only on with Historical Provider,

Source Information

Denese Earl Caranay | Baic Path